Supreme Court, U.S.
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No. 97-53

IN THE
Supreme Court of the United States
OCTOBER TERM, 1998

JANE M. ROBERTS,
Guardian for Wanda Y. Johnson,
Petitioner,

v.

GALEN OF VIRGINIA, INC.,
d/b/a University of Louisville Hospital,
Respondent.

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SUPREME COURT, U.S.

On Writ of Certiorari to the United States Court of Appeals for the Sixth Circuit

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BRIEF OF RESPONDENT

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QUESTION PRESENTED

Whether a patient may recover under the Emergency Medical Treatment and Active Labor Act, 42 U.S.C. § 1395dd, where she was appropriately screened by respondent's emergency room as having an emergency medical condition consisting of life-threatening trauma injuries, admitted to the hospital where the emergency condition was stabilized, treated for over two months, and then transferred to a skilled nursing facility for rehabilitation services while taking antibiotics for a urinary tract infection or possible pneumonia.

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On Writ of Certiorari to the United States Court of Appeals for the Sixth Circuit

BRIEF OF RESPONDENT

INTRODUCTION

The Emergency Medical Treatment and Active Labor Act, 42 U.S.C. § 1395dd ("EMTALA"), has spawned an extraordinary number of federal appellate court decisions. What is particularly striking is the number of those cases in which the courts of appeals have affirmed judgments in favor of the hospital, but felt compelled to do so on the basis of a legal theory different from the one employed by a lower court.[1]

[1] *See, e.g., Bryan* v. *Rectors and Visitors of Univ. of Va.*, 95 F.3d 349, 350-51 (4th Cir. 1996); *Eberhardt* v. *City of Los Angeles*,

The reason for this unusual phenomenon is not particularly difficult to ascertain: the lower courts are struggling with the inherent tension between the language of the statute—which at best only inartfully addresses the core concern that animated Congress's passage of EMTALA, viz., to prevent the practice of patient "dumping"—and the absence of any proof that Congress meant to supersede the States' traditional plenary authority over medical malpractice. Thus, the lower courts have grasped at a variety of limiting principles, some of which have more support in the statute than others.

In this case the Sixth Circuit has accomplished what is unquestionably the correct result. It is utterly counter-intuitive to imagine that Congress meant for EMTALA to apply to a transfer occurring months after a patient's admission to a hospital simply because the patient was suffering from a urinary tract infection at the time the patient was transported to a skilled nursing facility, which was plainly competent to treat this relatively minor condition. However, the Sixth Circuit reached this result by selecting a theory that, while conceptually consistent with Congress's desire to proscribe patient transfers that are made for financial or other non-medical reasons, is not supported by the language and structure of the statute. The Sixth Circuit, following its own prior precedent, identified petitioner's failure to prove that the transfer decision was prompted by an improper motive as an element of the EMTALA cause of action. After a careful review of the language and structure of the statute, respondent cannot defend the court of appeals' rationale.

That should not, however, be the end of the Court's inquiry. This is the first case this Court has agreed to

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62 F.3d 1253, 1257 (9th Cir. 1995); *Brooks* v. *Maryland Gen. Hosp., Inc.*, 996 F.2d 708, 709-10 (4th Cir. 1993); *Gatewood* v. *Washington Healthcare Corp.*, 933 F.2d 1037, 1040-41 (D.C. Cir. 1991); *Cleland* v. *Bronson Health Care Group, Inc.*, 917 F.2d 266, 268 (6th Cir. 1990).

review arising under EMTALA. Moreover, the United States, in its amicus brief, has put forth a strikingly provocative argument, i.e., EMTALA creates a federal medical malpractice standard that applies in this case and petitioner's claim should be allowed to go to trial on the question whether respondent failed to comply with that federal standard of care. In sum, the United States has addressed this case with a much broader brush than petitioner and has urged the Court to adopt a sweeping interpretation of EMTALA.

Respondent urges the Court to follow the unanimous decisions of the courts of appeals that have held that EMTALA is not a federal malpractice statute. Moreover, this case remains a suitable vehicle for the Court to recognize that there are significant limitations on the scope of a hospital's duties under EMTALA. At a minimum, the Court should consider the consensus opinion of the courts of appeals that a transfer or discharge by a hospital upon physician's orders, if the physician lacks actual knowledge that the patient has an emergency medical condition, does not violate EMTALA. The lower courts unanimously hold that actual knowledge is required. In this case, it is undisputed that the medical personnel at the hospital believed that petitioner's condition was stabilized at the time she was transferred. Certainly, no physician at the hospital perceived that respondent's medical condition constituted any type of "emergency." Thus, the question whether a physician's actual knowledge is an element of the plaintiff's claim is an important one that the Court should resolve in this case, where the evidence is uncontested that the patient's treating physicians did not consider her infection to constitute a serious problem.

Alternatively, the Court should consider the holdings of the Fourth and Tenth Circuits that EMTALA does not apply to treatment and discharge decisions occurring after a patient's initial screening and stabilizing treatment.

It is clear that the Sixth Circuit could have applied precisely this rationale to this case and the judgment would be unassailable. If that interpretation of EMTALA is correct, then this Court should recognize this alternative ground for affirmance and make clear that the United States is simply wrong in urging the Court to interpret EMTALA as a sweeping federal malpractice statute that applies to every discharge or transfer decision.[2]

In sum, respondent realizes that the Court granted the petition in this case with an intention to resolve merely the issue of whether a requirement of proof of illicit motive is an element of an EMTALA claim. To be sure, the Court can set aside the judgment on that basis. If the Court goes no further, then it probably will not significantly impede the vital progress the lower courts have made in confining EMTALA to its limited purpose of

[2] By identifying two alternative grounds for affirmance in this case, respondent does not mean to suggest that they are the only possible limiting principles for EMTALA. The courts of appeals have identified several others. For example, several courts of appeals apply a "disparate treatment" test to determine whether a patient received appropriate screening under EMTALA. Under this test, a hospital violates EMTALA's screening obligation only when it fails to apply its screening procedures uniformly to all patients who are similarly situated. *See, e.g., Marshall* v. *East Carroll Parish Hosp.*, 134 F.3d 319, 322-23 (5th Cir. 1998); *Summers* v. *Baptist Med. Ctr. Arkadelphia*, 91 F.3d 1132, 1138 (8th Cir. 1996); *Vickers* v. *Nash Gen. Hosp., Inc.*, 78 F.3d 139, 143-45 (4th Cir. 1996); *Correa* v. *Hospital San Francisco*, 69 F.3d 1184, 1192 (1st Cir. 1995); *Eberhardt* v. *City of Los Angeles*, 62 F.3d 1253, 1257-58 (9th Cir. 1995); *Holcomb* v. *Monahan*, 30 F.3d 116, 117 (11th Cir. 1994); *Repp* v. *Anadarko Mun. Hosp.*, 43 F.3d 519, 521-22 (10th Cir. 1994); *Gatewood* v. *Washington Healthcare Corp.*, 933 F.2d 1037, 1041 (D.C. Cir. 1991).

In an appropriate case, application of the "disparate treatment" test—or other limiting principles—might be appropriate. That issue was not litigated here because the parties agree that respondent provided an appropriate medical screening in this case. In contrast, the two alternative grounds proposed by respondent were raised and hence are legitimately posed here.

ensuring that all federally-funded hospitals are accessible to patients requiring emergency medical care. If, however, the Court were to entertain the United States' more aggressive interpretation of EMTALA, then respondent urges the Court to recognize that the judgment below is fully consistent with other courts of appeals' interpretations of this statute and thus there are alternative grounds available for affirmance that will permit this Court to provide important guidance to lower courts that now routinely face EMTALA claims whenever a medical malpractice action is asserted for allegedly negligent treatment arising in the hospital setting.

STATEMENT OF THE CASE

Statutory Background

Congress enacted EMTALA to prevent hospitals from "dumping patients," that is, from transferring patients upon physician's orders to other institutions for economic or other non-medical reasons. The problem of patient dumping became acute in the 1980's for two reasons. First, the number of Americans who lacked health insurance grew rapidly from the late 1970's through the mid-1980's. H.R. Rep. No. 100-531, at 7 (1988). Second, by reforming the Medicare payment system in 1983, Congress eliminated the ability of hospitals to pass on to the government the costs of caring for indigent patients. *Id.* Given these systemic incentives for hospitals not to admit indigent or uninsured patients, Congress concluded that federal legislation was necessary because "28 States [had] no" laws that would "ensure that no emergency patient is denied emergency care because of inability to pay." 131 Cong. Rec. S 13,904 (daily ed. Oct. 23, 1985) (statement of Sen. Kennedy).

The Act responds to the patient-dumping problem by requiring hospitals to provide patients for whom treatment is requested in an emergency room with (a) an appropriate medical screening examination, and with either

(b) stabilizing treatment of the emergency medical condition or (c) an appropriate transfer to another medical facility for such treatment. 42 U.S.C. §§ 1395dd(a), (b), (c). The Act provides for multiple methods of enforcement, and thus Congress did not look to private individuals as the primary enforcers of its ban on patient dumping. First, the Secretary of Health and Human Services can impose civil money penalties on participating hospitals and physicians who negligently violate any requirement of the Act. 42 U.S.C. § 1395dd(d)(1). Any physician who commits a violation that is "gross and flagrant or is repeated" also can be excluded by the Secretary from participation in Medicare and state health care programs. 42 U.S.C. § 1395dd(d)(1)(B). Second, any individual who suffers harm as a direct result of a violation by a participating hospital can bring a private civil action to recover damages for personal injury that are available under state law. 42 U.S.C. § 1395dd(d)(2)(A). Third, a medical facility, such as a transferee hospital, that suffers a financial loss as a direct result of a violation by a participating hospital also can bring a civil action to recover damages that are available under state law. 42 U.S.C. § 1395dd(d)(2)(B). The relevant provisions of EMTALA are reprinted in the appendix to the petition for certiorari. Pet. App. A55-A66.

Factual Background

Wanda Johnson was severely injured when she was struck by a motor vehicle on May 9, 1992. She was taken to the emergency room of Humana Hospital University of Louisville ("Humana"), which is a teaching hospital affiliated with the University of Louisville. Pursuant to an arrangement with the Commonwealth of Kentucky, the City of Louisville and Jefferson County, Kentucky, Humana is obligated to provide emergency and other treatment to all individuals who request it, regardless of ability to pay. This arrangement continues the

University of Louisville's historic role of ministering to the health care needs of indigent patients as a public service in the course of its teaching programs. Indeed, Humana is the primary facility in the Louisville area that provides emergency care to indigent individuals.

Humana's emergency room physicians diagnosed Ms. Johnson as having life-threatening injuries, and admitted her to the hospital. Ms. Johnson was diagnosed and treated for multiple fractures to her pelvis, right leg and four vertebrae of her spine. She also suffered a serious head injury. During surgery Ms. Johnson's spleen was removed and she was connected to a ventilator. Her condition was stabilized and she continued to be treated by the hospital for over two months, during that time she was removed from the ventilator.

By July, Ms. Johnson was "pretty much on a steady course of showing improvement." J.A. 117 (Deposition of Walid Abou-Jaoude, M.D. ("Abou-Jaoude Dep."), R. 67, at 17).[3] Indeed, she was "stable" throughout July and was moved to a "step-down unit" within the hospital (a unit that involves "less intensive" care) a couple of weeks before her discharge. *Id.* at 116-18 (Abou-Jaoude Dep. at 15, 17). Ms. Johnson suffered recurrent pneumonia and urinary tract infections during her stay at Humana as a result of being on a ventilator and having a chronic indwelling catheter, but these conditions were successfully treated with antibiotics and did not cause her health to become unstable. Abou-Jaoude Dep. at 14-15, 17-18.

When Ms. Johnson's doctors—who are independent contractors and not employees of Humana—made the medical determination that she was "stable enough to be transferred" and "making progress," they instructed the

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[3] The Joint Appendix is cited as "J.A. ——." The Appendix to the Petition for Certiorari is cited as "Pet. App. ——." Deposition transcripts and other record materials are identified by their docket number in the record.

social workers and discharge planners at Humana to search for a suitable place for her at a skilled nursing home so that she could continue her recovery and receive rehabilitation therapy. J.A. 118-20 (Abou-Jaoude Dep. at 19-21) and Abou-Jaoude Dep. at 18, 22; see also Deposition of Frank Miller, M.D. ("Miller Dep."), R. 53, at 26 (the decision to discharge a patient from Humana hospital is always a medical decision made by physicians). These efforts were complicated by "a long-standing shortage of nursing home beds in Kentucky." J.A. 137 (Affidavit of Nancy Fred ("Fred. Aff.) ¶ 3). They also were complicated by the fact that repeated attempts to involve Ms. Johnson's family in the discharge planning process proved unsuccessful. Nancy Fred, the social worker in charge of Ms. Johnson's discharge plan, repeatedly tried to contact Ms. Johnson's family because she recognized that it was "real important" to "reach the next of kin" and keep them apprised of the progress of the discharge plan. J.A. 25, 35 (Deposition of Nancy Fred ("Fred Dep."), R. 10, at 30, 42). However, Ms. Johnson's mother "didn't seem to want to be involved in any decisions." J.A. 35 (Fred Dep. at 42). Similarly, Ms. Johnson's husband could not be located by Ms. Fred, or indeed by other family members, at relevant decision points. J.A. 19, 34-35 (Fred Dep. at 25, 42). Because "everybody else had absconded or eliminated themselves," Ms. Fred communicated with Ms. Johnson's aunt, petitioner Jane Roberts, as "the only family member left that [the hospital] could contact." J.A. 35-36 (Fred Dep. at 42-43).

In late July, Ms. Johnson was accepted for admission at Crestview Health Care Center ("Crestview"), a state-licensed skilled nursing home in Indianapolis, Indiana. Ms. Fred had expanded her search into Indiana, which is just across the river from Louisville, because of the shortage of facilities in Kentucky. J.A. 137 (Fred Aff. ¶ 3). Humana determined that Crestview was an appropriate placement for Ms. Johnson based on Ms. Fred's discussions with Jo Anna Nolte, a representative of Crestview, who stated that Crestview "was a skilled nursing facility, certified by the state certification agency; that they had the therapies that Mrs. Johnson could benefit from to work toward her rehabilitation," including speech therapy, physical therapy, and occupational therapy. J.A. 31-32 (Fred Dep. at 36-37); see also Deposition of Jo Anna Nolte ("Nolte Dep."), R. 18, at 61 (Crestview was certified by the State of Indiana to provide skilled nursing care, including to patients who required "heavy care"); Fred Dep. at 49 (Humana's "main consideration" was that the receiving facility be "certified for skilled nursing"). Ms. Fred also spoke "several times with Randy Gross, who was the director/administrator at Crestview." *Id.* at 38-39. She also discussed the prospective transfer with Dr. Abou-Jaoude, who "[made] sure [Crestview] was a skilled nursing facility." J.A. 121-22 (Abou-Jaoude Dep. at 29-30); J.A. 38-39 (Fred Dep. at 54-56).[4]

Ms. Fred attempted to contact Ms. Johnson's husband to discuss the potential transfer to Crestview, but was unsuccessful. J.A. 25-26, 34-35 (Fred Dep. at 31, 41-42). She then contacted petitioner, Ms. Johnson's aunt, who did not object to the transfer. J.A. 34-36 (Fred Dep. at 41-43). If petitioner *had* opposed the transfer, "[Humana] wouldn't have discharged [Ms. Johnson] to that facility." J.A. 36 (Fred Dep. at 43).

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[4] Ms. Nolte testified that Ms. Fred told her that "she was getting a lot of pressure to discharge [Ms. Johnson] due to they knew they weren't going to get paid for this person." J.A. 42 (Nolte Dep. at 23); *see also* Nolte Dep. at 48 (Ms. Fred indicated that the alleged pressure was from the hospital, not from Ms. Johnson's physicians). Ms. Fred denies that she made any such statements to Ms. Nolte, and has stated that she "was under no pressure from any department of the hospital or physician to locate a nursing home placement for Wanda Johnson for financial reasons." J.A. 137-38 (Fred Aff. ¶¶ 5-7).

Ms. Johnson was "stable" during the two weeks prior to her discharge and certainly suffered from no emergency medical condition. J.A. 123 (Abou-Jaoude Dep. at 37). She developed a fever the day before the transfer, just as she had several times during her stay in the hospital, and her doctors performed their "routine workup," which included a chest x-ray, urinalysis, and blood work. J.A. 126-27 (Abou-Jaoude Dep. at 48-49). Because the urinalysis was cloudy and she had an elevated white blood cell count, the doctors suspected that she had yet another urinary tract infection. *Id.* They also suspected that she might have another bout of pneumonia based on a bronchial specimen. J.A. 128 (Abou-Jaoude Dep. at 50-51). They put her on Bactrim, a broad spectrum antibiotic, which treats urinary tract infections and had been used successfully to treat Ms. Johnson's pneumonia previously. *Id.* According to Dr. Abou-Jaoude, none of Ms. Johnson's doctors ever considered delaying the transfer because of the suspected urinary tract infection. J.A. 130 (Abou-Jaoude Dep. at 53).

Ms. Johnson was transferred to Crestview on July 24, 1992. All of her doctors concluded that she was sufficiently stable to warrant transfer on that date. J.A. 131 (Affidavit of David Richardson, M.D. ("Richardson Aff.") ¶ 5); J.A. 133 (Affidavit of Walid Abou-Jaoude, M.D. ("Abou-Jaoude Aff.") ¶ 4); J.A. 135 (Affidavit of Brian Harbrecht, M.D. ("Harbrecht Aff.") ¶ 4). Of even greater importance in connection with this case, it was plain that "she did not have an emergency medical condition on the date of her transfer." J.A. 103 (Miller Dep. at 30). Moreover, the undisputed evidence in this case is that urinary tract infections are routinely treated at skilled nursing facilities, including Crestview, and even can be treated on an outpatient basis. See, e.g., J.A. 83-84 (Deposition of Kambiz T. Karimi, M.D., R. 89, at 16); J.A. 87 (Deposition of Taiseer Shatara, M.D. ("Shatara Dep."), R. 90, at 20) (Crestview was capable

of managing a patient with a urinary tract infection and could modify antibiotic therapy if necessary); J.A. 110 (Deposition of John W. Stuy, M.D., Mar. 28, 1995 ("Stuy Dep."), R. 88, at 25). The discharge summary that Dr. Abou-Jaoude prepared for Ms. Johnson identified all of the medications that she was taking, including Bactrim for a urinary tract infection. J.A. 124-25 (Abou-Jaoude Dep. at 46-47); Ex. 4 to Abou-Jaoude Dep., at 4.

At the time of Ms. Johnson's discharge, her application for social security disability, including Kentucky Medicaid, was pending before the state agency. J.A. 39 (Fred Dep. 55-56); J.A. 137 (Fred Aff. ¶ 4). The physician who signed the discharge papers, however, was unaware of her financial status. J.A. 133 (Abou-Jaoude Aff. ¶ 5). Indeed, physicians at Humana Hospital "are typically not aware of a patient's payment source at the time of discharge." J.A. 135 (Harbrecht Aff. ¶ 5); J.A. 104 (Miller Dep. at 31-32). Moreover, as was true of Ms. Johnson, Humana Hospital often does not know a patient's ultimate ability to pay at the time of discharge because insurance coverage issues, public assistance applications, and lawsuits can take months to resolve, particularly in accident cases. *Id.;* see also Deposition of John Nord, R. 47, at 15-16 (it can take up to two years for a Medicaid application to be approved). Accordingly, the undisputed evidence is that Ms. Johnson's financial situation played no part in her physicians' medical decision to transfer her to a skilled nursing facility. J.A. 131 (Richardson Aff. ¶ 6); J.A. 133 (Abou-Jaoude Aff. ¶ 6); J.A. 135 (Harbrecht Aff. ¶ 6).

Ms. Johnson arrived at Crestview at approximately 5:30 p.m. on July 24, and was assessed by Charna Treser, the licensed practical nurse on duty. At approximately 6:00 p.m., Ms. Treser called Dr. Taiseer Shatara, M.D., who authorized Ms. Johnson's admission based on the information provided by Ms. Treser and the physician's con-

clusion "that [Ms. Johnson] was stable and that she was an appropriate candidate for nursing home placement." J.A. 88 (Shatara Dep. at 21); see also J.A. 85 (Shatara Dep. at 17) ("[B]ased on the information that was given to me at the time, I felt that the patient was stable enough"). Ms. Johnson also was seen that night by Lynnette Ferguson, Crestview's director of nursing, who noted that Ms. Johnson had an elevated temperature of 100.5 degrees at the time of her arrival, but did not consider this unusual for a new admission. Deposition of Lynnette Ferguson ("Ferguson Dep."), R. 19, at 24-25, 56. Ms. Ferguson concluded that Ms. Johnson was an appropriate patient to be cared for at Crestview. *Id.* at 52. When Ms. Treser completed her shift at 11:00 p.m., she recorded on her chart "that there were no signs or symptoms of distress for Ms. Johnson." J.A. 59 (Deposition of Charna Treser ("Treser Dep."), R. 92, at 69.

According to Ms. Johnson's chart, her temperature was next taken at 9:10 a.m. on July 25, at which time it was 105 degrees. Ferguson Dep. at 56. Ms. Ferguson contacted one of Crestview's doctors because of the elevated temperature and because Ms. Johnson was experiencing respiratory distress. *Id.* at 26. The doctor instructed her to send Ms. Johnson to the emergency room of Midwest Hospital ("Midwest") for an evaluation. *Id.* at 26-27. Ms. Johnson was admitted to Midwest and remained there for several months. The physician who admitted her, Dr. John W. Stuy, opined that the cause of her deterioration while at Crestview was "[e]ither the urinary tract infection that she was treated for was not covered, was not sensitive to the antibiotic that she was sent on transfer, or she developed another acute pulmonary infection." J.A. 112 (Stuy Dep. at 32). Dr. Stuy conceded that Ms. Johnson's condition was "substantially worse" when she was admitted to Midwest Hospital than when she had been discharged from Humana under physician's orders. Stuy Dep. at 76, 91-92. He also conceded that the transfer itself had not caused her to become ill. *Id.* at 75-76. His opinion was that she would have developed the same infection at Humana and at Crestview, but that the problem likely would have been addressed earlier in the hospital setting. *Id.* at 92-93.

Procedural History

On August 30, 1993, petitioner Jane Roberts, who is now Ms. Johnson's guardian, filed an action on her behalf under EMTALA, 42 U.S.C. § 1395dd, alleging that Humana violated EMTALA by transferring Ms. Johnson to Crestview pursuant to physician's orders when her condition was not stable. The complaint does not allege that Ms. Johnson's initial screening at Humana was inappropriate under EMTALA and the parties agree that Ms. Johnson had an emergency medical condition when she arrived at Humana's emergency room. The complaint also alleged that Humana violated the due process and equal protection clauses of the United States and Kentucky constitutions and was liable under state law for the negligence of Ms. Johnson's doctors. The Secretary of Health and Human Services did not seek any civil money penalties against either the hospital or its physicians in this case. Nor did Midwest Hospital, the downstream provider, assert its statutory right to bring a civil action against Humana to recover its costs of treating Ms. Johnson.

A. Humana moved for summary judgment in the district court. The district court initially denied Humana's motion with respect to the EMTALA claim. Pet. App. A26-A45. The court held that there was a genuine issue of material fact as to whether Ms. Johnson was stabilized before she was transferred to Crestview, based on the conflicting testimony of Ms. Johnson's treating physicians at Humana and the physician who later treated her at Midwest Hospital. *Id.* at A29-A36. The court also held that there was a genuine issue of material fact as to

whether Humana acted with an improper motive, based on the conflicting testimony of the Humana social worker and the Crestview representative concerning whether the social worker had indicated that she was under pressure to arrange a transfer for Ms. Johnson because of her financial status. *Id.* at A37-A39. The court relied on the Sixth Circuit's decision in *Cleland* v. *Bronson Health Care Group, Inc.*, 917 F.2d 266, 271 (6th Cir. 1990), for its conclusion that "the presence of an improper motive [is] an essential element in a claim for relief under [EMTALA]." Pet. App. A37. The court granted Humana's motion for summary judgment on the constitutional and state law negligence claims.

Humana moved for reconsideration on the EMTALA claim, arguing that there was no genuine issue of material fact as to causation because, irrespective of any improper motive on the part of the hospital, the decision to discharge Ms. Johnson was made by physicians who were not aware of her financial status. The motion for reconsideration was granted and the court entered judgment for Humana on the EMTALA claim. Pet. App. A48-A52.

B. Petitioner appealed the district court's ruling on the EMTALA and state law negligence claims and the Sixth Circuit affirmed. Pet. App. A1-A25. The court noted that there was a genuine issue of material fact as to whether Ms. Johnson was stable when transferred. *Id.* at A13. However, it held that the district court "properly interpreted the *Cleland* holding as requiring that a plaintiff prove a hospital acted with an improper motive in order to recover under the EMTALA." *Id.* at A10. Although *Cleland* involved a claim of inappropriate screening, the court held that proof of improper motive is required "to make a case of inappropriate screening *or improper discharge*" under EMTALA. *Id.* at A11-A12 (emphasis added). Applying the improper motive test

to the facts of this case, the court noted that although there was evidence that "some representative of the hospital may have been acting with an improper motive," there was no genuine issue of material fact with respect to causation because "there is no evidence that this improper motive caused Dr. Abou-Jaoude to issue Johnson's discharge order." *Id.* at A15. Accordingly, the court of appeals affirmed the summary judgment on the EMTALA claim. *Id.* at A16. It also affirmed the summary judgment on the state law negligence claim, which is not at issue here.

SUMMARY OF ARGUMENT

A. The first sentence of the United States' brief (U.S. Br. 1) accurately describes what this case is about: it "concerns the requirements for establishing liability under the Emergency Medical Treatment and Labor Act (EMTALA), 42 U.S.C. § 1395dd." The United States, however, offers a theory of EMTALA liability that extends well beyond the narrow question directly posed by the Sixth Circuit's holding that EMTALA liability requires proof of improper motive in transferring or discharging a patient. The government argues that the statute imposes a federally-defined "objective standard of [medical] care," against which the federal courts should assess the adequacy of treatment provided to stabilize a patient's "medical condition." U.S. Br. 11. The government's response to the court of appeals' effort (shared by every other court of appeals that has addressed the issue) to prevent EMTALA from becoming "nothing more than a federal remedy for medical malpractice," (Pet. App. A12) is to assert that its opinion merely "represents a disagreement with Congress's decision" and is thus "of little import." U.S. Br. 13.

Under the government's apparent view of the statute, once a patient is brought into the hospital for an emergency condition, the hospital assumes a federal duty under EMTALA to ensure the stability of the patient's "medical

condition" prior to his or her release or transfer from the hospital under physician's orders. In embracing this expansive interpretation of the statute, the United States' brief illustrates why this Court should take advantage of this opportunity to resolve the critical and recurring question regarding the breadth of EMTALA.

The issue of whether EMTALA can be more reasonably confined than the Solicitor General proposes is critical because, if Congress has created a "federal malpractice statute" regulating all discharges by hospitals pursuant to physician's orders, then the federal government has asserted extraordinary authority to regulate the practice of medicine in the hospital setting. This is the type of arrogation of power by the federal government in an area traditionally regulated by the States where this Court has insisted that Congress evince its intention in language that admits of but one conclusion. See, e.g., *United States* v. *Bass*, 404 U.S. 336, 349 (1971).

In considering whether EMTALA has such a breathtaking sweep or is instead a gap-filling statute designed to impose novel obligations on hospitals—not to supersede state-law imposed responsibilities—the Court must examine the language of the statute against *Bass*'s fundamental rule of construction derived from principles of federalism. Judged against those standards, it is clear that EMTALA is not what the Solicitor General asserts it is. The language, structure and history of this statute reveal that it is not a revolutionary enactment; to the contrary, it imposes discrete and relatively narrow duties and is intended to interfere as little as possible with the operation of state law. See 42 U.S.C. § 1395dd(f).

B. For these reasons, the Court should reject the government's suggestion that EMTALA must be read expansively. Nevertheless, simply rejecting the government's interpretation will not resolve the question of how EMTALA should be applied in this particular case. To its credit, the court of appeals sought to impose a reasonable limitation on the statute that would protect patients from being "dumped" by hospitals for non-medical reasons, but still would ensure that EMTALA does not supersede state malpractice law in hospitals. The problem is that while the "motive" test adopted by the court below is consistent with Congress's obvious intent, it lacks support in any of the traditional sources of statutory construction. To be sure, it is a sensible solution to a difficult issue, but it is not the solution that Congress expressed in the statutory text. Respondent and the health care industry would welcome this rule of law, but neither can credibly argue that the Court should read it into the statute.

To the extent the Court accepts respondent's concession, it may set aside the judgment below on the ground that the statute will not bear the Sixth Circuit's construction. Simply setting aside the judgment below, however, would carry with it significant opportunity costs. This case squarely presents two alternative arguments for affirmance that have been adopted by other courts of appeals. This Court should address those alternative arguments for affirmance in order to spare patients and hospitals the necessity of expending scarce resources in litigating the bounds of EMTALA in cases that arise every day.

First, the record in this case is absolutely clear that all of the physicians involved with Ms. Johnson's treatment believed that her condition had stabilized and did not regard her urinary tract infection as a serious, much less an "emergency," medical condition. Thus, under the law in the Fourth, Fifth, Eighth, Ninth, Tenth and D.C. Circuits, all of which require an EMTALA plaintiff to prove that the hospital had actual knowledge of the patient's emergency condition before any duty arises under EMTALA, summary judgment in favor of the hospital was clearly appropriate in this case. Because that interpretation of the statute is compelled by the language,

structure and history of EMTALA, the Court should apply it in this case as an alternative ground to affirm.

Second, there is no question that the hospital in this case properly screened Ms. Johnson and fully treated the emergency condition caused by her tragic accident. The issue then is whether EMTALA imposes any continuing duties on the hospital once the emergency treatment has been completed. Under the law in the Fourth and Tenth Circuits, it is clear that the hospital's duties under EMTALA lapsed well before her transfer pursuant to physician's orders to a skilled nursing facility and that state tort law became the sole source of potential liability. Again, this alternative basis for affirmance is grounded in the language and purpose of the Act and thus the Court should apply it here rather than prolong a lawsuit that has no merit.

Review of these alternative arguments is particularly warranted given the government's position that it has broad authority to regulate hospital treatment decisions under federal standards. If that interpretation is wrong—and respondent believes strongly that it is—then the Court should act promptly before the bureaucratic machinery of the Department of Health and Human Services undertakes to enforce its vision of EMTALA. At a minimum, the Court should make clear that it was not Congress's expressed intent in enacting EMTALA to occupy the field of medical malpractice in the emergency hospital setting.

ARGUMENT

It is common ground among the parties and their respective *amici* that the immediate focus of Congress in enacting EMTALA was to prevent the practice of patient dumping. Congress was rightly offended that some hospitals would turn individuals away without providing any initial examination or without providing immediate treatment when indicated for reasons having nothing to do with the patient's medical condition. See H.R. Rep.

No. 99-241, pt. 1, at 27 (1985), *reprinted in* 1986 U.S.C.C.A.N. 42, 605. Respondent is acutely aware of the problems that animated Congress's action because the University of Louisville Hospital has for years been the transferee hospital for hundreds, if not thousands, of patients that other hospitals refused to admit and to treat. As in this case, respondent has always admitted all patients, screened them, treated them, and when in the judgment of the treating physicians the time was right, arranged for the patients' discharge or their transfer to other, more appropriate treatment settings.

Where the parties disagree is in determining just how broadly Congress in enacting EMTALA imposed duties on essentially all of the nation's hospitals.[5] The United States asserts broadly that "the hospital's duty is to provide sufficient treatment 'to stabilize the medical condition,' " of the patient. U.S. Br. 11. This is a duty in the context of this case that apparently extends months after the patient is admitted to the hospital and has received extraordinary medical care. Moreover, this duty apparently applies to a discharge decision made by physicians who unanimously agree that the patient no longer faces a medical emergency. The unmistakable effect of this argument is that federal law overlays and effectively supersedes state law with respect to much, if not most, medical treatment decisions arising out of an emergency care patient's transfer or discharge from a primary care facility.

Without question, Congress has the authority to achieve this objective under its Commerce and Spending powers, but it is difficult to read 42 U.S.C. § 1395dd(b)(1), which is the only portion of this statute directly at issue here, to accomplish the sweeping effect the United States proposes. The statute is limited by its terms to the situa-

[5] The Solicitor General suggests that EMTALA applies to 99 percent of all hospitals. U.S. Br. 2 n.1. Respondent does not doubt the accuracy of that estimate.

tion where an individual "comes to a hospital" and the hospital, after a mandatory screening examination, actually "determines" that "an emergency medical condition" exists. In that precise circumstance EMTALA requires the individual's medical condition to be "stabilize[d]" through medical treatment.

Although this language cannot be read to include an element of "motive" as a requirement for liability, neither can it be stretched as the United States would read it to include a roving commission to the Department of Health and Human Services to seek out transfers and discharges of patients that failed to meet a "federal" standard of care, even though the physicians believed that there was no serious, much less emergency, condition present in the patient and even though the patient had been admitted and treated at the hospital for more than two months.

What makes the United States' effort to read so much into this statute particularly troublesome is its failure even to acknowledge the serious violence to principles of federalism that is embodied in its approach. The cavalier suggestion that these concerns are "of little import" (U.S. Br. 13) seems hollow given the lack of clear evidence of congressional intent, which this Court requires before assuming that traditional state regulation will be superseded by federal authority.

Accordingly, respondent begins where the Solicitor General declines to tread—an analysis of the relevance of the fundamental canon of statutory construction that requires clear intent by Congress before the Court radically alters the "balance of power" in an area, like medical malpractice, that has historically been the exclusive preserve of the States. Then, respondent will show that the United States' argument does serious harm to those principles and thus should be rejected. Respondent next will examine the Sixth Circuit's well-meaning, but ultimately unsupportable effort to maintain the proper balance of power between federal and state law in the emergency care setting. Respondent will offer two alternative rationales that the Court should consider, both of which would have been employed by more than one other court of appeals and both of which would unquestionably warrant affirmance of the judgment in this case. By entertaining these two arguments, the Court will provide important guidance to the federal courts and prevent EMTALA from allowing every run-of-the-mill malpractice action arising in the hospital setting from becoming a lawsuit in federal court.

I. EMTALA DOES NOT FUNDAMENTALLY ALTER THE CONSTITUTIONAL BALANCE BETWEEN FEDERAL AND STATE RELATIONS BY CREATING A FEDERAL STANDARD OF CARE FOR MEDICAL DIAGNOSIS AND TREATMENT FOR ALL PATIENTS WHO ENTER A HOSPITAL'S EMERGENCY ROOM.

A. When construing a federal statute such as EMTALA, ordinarily it is appropriate to start with the language and structure of the statute, and then construe that language using other traditional tools of statutory interpretation. In this case, however, it is important to take a step back and consider some first principles because Congress was not writing EMTALA on a blank slate. Rather, Congress undertook to legislate in an area —the physician-patient relationship—that traditionally has been regulated exclusively by state law. Accordingly, this case implicates fundamental principles of federalism that must be examined in order to put the task of interpreting EMTALA into its proper context.

This Court has long followed the "ordinary rule of statutory construction that if Congress intends to alter the 'usual constitutional balance between the States and the Federal Government,' it must make its intention to do so 'unmistakably clear in the language of the statute.' " *Will* v. *Michigan Dept. of State Police*, 491 U.S. 58, 65

(1989), quoting *Atascadero State Hospital* v. *Scanlon*, 473 U.S. 234, 242 (1985). This principle, which is deeply rooted in concepts of federalism, is "nothing more than an acknowledgment that the States retain substantial sovereign powers under our constitutional scheme, powers with which Congress does not readily interfere." *Gregory* v. *Ashcroft*, 501 U.S. 452, 461 (1991). Although Congress may alter the balance of federal-state power, "it is incumbent upon the federal courts to be certain of Congress' intent before finding that federal law overrides" state sovereignty. *Atascadero*, 473 U.S. at 243.[6]

The Court has applied this clear statement requirement in determining whether Congress intended to expand the role of the federal government vis-a-vis the States in an area of traditional state responsibility. In *United States* v. *Bass*, 404 U.S. 336 (1971), for example, the Court narrowly construed the scope of a federal criminal statute because the broad reading would have "render[ed] traditionally local criminal conduct a matter for federal enforcement." *Id.* at 350. The Court invoked the principle that "unless Congress conveys its purpose clearly, it will not be deemed to have significantly changed the federal-state balance" in an area of traditional state concern such as criminal jurisdiction. *Id.* at 349. The Court explained that "[i]n traditionally sensitive areas, such as legislation affecting the federal balance, the requirement of a clear statement assures that the legislature has in fact

[6] This clear statement rule is consistent with the principle used in preemption cases that where intrusion into an area of traditional state responsibility is involved, the Court will narrowly construe the preemption provision to go no further than necessary to effectuate the federal purposes. *See, e.g., Cipollone* v. *Liggett Group, Inc.*, 505 U.S. 504, 518 (1992) (plurality) ("[W]e must construe these provisions in light of the presumption against the pre-emption of state police power regulations."); *Rice* v. *Santa Fe Elevator Corp.*, 331 U.S. 218, 230 (1947) (this Court "start[s] with the assumption that the historic police powers of the States were not to be superseded by the Federal Act unless that was the clear and manifest purpose of Congress").

faced, and intended to bring into issue, the critical matters involved in the judicial decision." *Id.*

As this Court has recognized, tort law, like criminal law, is a classic area of traditional state responsibility. See, e.g., *Silkwood* v. *Kerr-McGee Corp.*, 464 U.S. 238, 248 (1984) (acknowledging "the States' traditional authority to provide tort remedies to their citizens"). Moreover, the primacy of the States in addressing matters of public health and safety repeatedly has been recognized by this Court. See, e.g., *Medtronic, Inc.* v. *Lohr*, 518 U.S. 470, 475 (1996) (noting "the prominence of the States in matters of public health and safety"); *Hillsborough County* v. *Automated Med. Labs., Inc.*, 471 U.S. 707, 719 (1985) ("[T]he regulation of health and safety matters is primarily, and historically, a matter of local concern"). Accordingly, because Congress drafted EMTALA against the backdrop of a well-established body of state tort law governing medical malpractice, the statute should only be read to expand the role of the federal government vis-a-vis the States in the area of the physician-patient relationship if the clear statement requirement is satisfied. Indeed, given the Court's consistent adherence to the clear statement requirement, it should be presumed that Congress was aware when it drafted EMTALA that any intent to "federalize" the area of medical malpractice law had to be clearly and unambiguously expressed in the statutory language.

B. The clear statement requirement provides the background against which the Court must evaluate the United States' provocative position that EMTALA "imposes an objective standard of care" for medical diagnosis and treatment, which requires a hospital to diagnose, treat and stabilize a "medical condition" once a patient is admitted for emergency care. U.S. Br. 11; see also *id.* at 12 (Congress chose to address the problem of patient dumping "through imposition of a substantive standard of medical care"). The essential flaw in the United States' argu-

ment is that none of the language of EMTALA comes close to satisfying the clear statement requirement. Congress simply provided no hint in the statutory language—much less an "unmistakably clear" statement—that it intended fundamentally to alter the federal-state balance by creating a substantive federal malpractice standard.

The Court therefore should reject the United States' sweeping interpretation of EMTALA—an interpretation that is not advanced by petitioner and, not surprisingly, has unanimously (and emphatically) been rejected by every court of appeals to consider it. See *Gatewood* v. *Washington Healthcare Corp.*, 933 F.2d 1037, 1041 (D.C. Cir. 1991) ("[W]e cannot agree that [EMTALA] creates a sweeping federal cause of action with respect to what are traditional state-based claims of negligence or malpractice"); *Correa* v. *Hospital San Francisco*, 69 F.3d 1184, 1192 (1st Cir. 1995), *cert. denied*, 517 U.S. 1136 (1996) ("EMTALA does not create a cause of action for medical malpractice"); *Vickers* v. *Nash Gen. Hosp., Inc.*, 78 F.3d 138, 142 (4th Cir. 1996) ("The other circuit courts are universally in accord on the need to distinguish EMTALA claims from standard claims of negligence and misdiagnosis"); *Marshall* v. *East Carroll Parish Hosp.*, 134 F.3d 319, 322 (5th Cir. 1998) ("We agree with the other courts which have interpreted EMTALA that the statute was not intended to be used as a federal malpractice statute"); *Cleland* v. *Bronson Health Care Group, Inc.*, 917 F.2d 266, 272 (6th Cir. 1990) (the language of EMTALA "precludes resort to a malpractice or other objective standard of care"); *Summers* v. *Baptist Med. Ctr. Arkadelphia*, 91 F.3d 1132, 1137 (8th Cir. 1996) ("[E]very court that has considered EMTALA has disclaimed any notion that it creates a general federal cause of action for medical malpractice in emergency rooms"); *Eberhardt* v. *City of Los Angeles*, 62 F.3d 1253, 1258 (9th Cir. 1995) ("The statutory language of the EMTALA clearly declines to impose on hospitals a national standard of care in screening patients"); *Repp* v.

Anadarko Mun. Hosp., 43 F.3d 519, 522 (10th Cir. 1994) ("[W]e believe that the language of section 1395dd(a) precludes the adoption of a standard tantamount to a federal malpractice statute"); *Holcomb* v. *Monahan*, 30 F.3d 116, 117 (11th Cir. 1994) ("Section 1395dd(a) is not designed to redress a negligent diagnosis by the hospital; no federal malpractice claims are created"). See also, *Owens* v. *Bellevue Hosp. Ctr.*, No. 95-9016, 1996 WL 134227 (2d Cir. Mar. 25, 1996) (unpub.) ("EMTALA is not intended to provide a federal remedy for medical malpractice"). The suggestion by the government that these rulings "merely represent[] a disagreement with Congress's decision" and therefore are "of little import" should be rejected in this Court. U.S. Br. 13.

The text of EMTALA contains no "clear statement" that Congress intended to create a substantive federal standard of care for the diagnosis and treatment of patients whenever they are transferred or discharged from a hospital under physician's orders. To the contrary, the language of 42 U.S.C. § 1395dd(b)(1) is extremely narrow. It does not purport to impose duties on the hospitals in general. The statute's duties are triggered only by having an individual "come[] to a hospital." Even then, the federal statute does not purport to impose broad responsibilities on the facility. Under subsection (a) a screening examination is required, but there is no dispute that a proper examination was made when Ms. Johnson "[came] to the emergency department" in critical condition. But the statute does not pointedly impose upon the hospital a duty to ascertain the patient's condition and treat it non-negligently. Instead, no treatment obligation arises at all unless and until the hospital actually "determines" that "an emergency medical condition" exists. Only then is there a duty to provide "such treatment as may be required to stabilize the medical condition." Of course, the condition that must be stabilized is not some

unknowable condition that may arise, but rather the specific "emergency medical condition" that is mentioned earlier in subsection (b).

Viewed against the existing common law obligations imposed by the States, it is easy to understand how this language can and should be read narrowly, not broadly as the United States asserts. First, the statute creates a duty to enter into a physician-patient relationship in the hospital setting. By requiring hospitals to accept patients for screening and diagnosis, federal law eliminated a loophole in state law that had allowed hospitals to turn away patients without having to confront possible malpractice claims. See *supra*, at 5. Second, the effect of creating the relationship, by itself, triggered state common law obligations to provide non-negligent diagnosis and care. There is nothing in the statute that remotely suggests Congress found state law inadequate on this count or that Congress felt any need to supplement, much less supplant, state law. Finally, the only real "treatment" duty was to "stabilize" the patient's *emergency* medical condition. There is no guarantee of a quality of care or the requirement of a continuing duty beyond what was necessary to get the patient past the immediate crisis. In sum, EMTALA is a focused statute that imposes limited responsibilities on hospitals and nothing in its language or purpose supports the suggestion that it imposes "a substantive standard of medical care" that arises regardless of the physician's determination of the patient's condition or regardless of whether the patient's emergency condition has long since lapsed.

The United States' principal textual support for its position is that Congress incorporated a "reasonableness" standard in some of the definitions of key terms that appear in EMTALA. U.S. Br. 16-18. For example, EMTALA defines "to stabilize" as to provide such medical treatment as may be necessary to assure, "within *reasonable* medical probability," that the patient will not

materially deteriorate. 42 U.S.C. § 1395dd(e)(3)(A) (emphasis added). Similarly, an "emergency medical condition" is defined as a condition "manifesting itself by acute symptoms of sufficient severity (including severe pain) such that the absence of immediate medical attention could *reasonably* be expected to result in" serious jeopardy to the patient's health or other adverse consequences. 42 U.S.C. § 1395dd(e)(1)(A) (emphasis added).

The short answer to the United States' argument is that Congress's use of the term "reasonable" in two definitions that appear in EMTALA falls far short of an "unmistakably clear" statement that Congress intended to create a federal standard of care and thereby inject federal courts into the difficult business of adjudicating all medical malpractice claims arising in a hospital setting. At a bare minimum, if Congress had intended to establish a federal standard of care, Congress would have addressed that issue directly in a substantive provision in the statute, rather than indirectly in two definitions.

Even if the statutory definitions are probative on this issue, they arguably cut the other way because EMTALA does not uniformly define a hospital's obligations in terms of "reasonableness." As the United States concedes, "the Act does not define the term 'appropriate medical screening examination'" at all. U.S. Br. 3. This omission is significant because if Congress had intended to enact a substantive standard of care, it presumably would have done so for hospitals' *screening* obligation as well as their *stabilization* obligation. Congress's failure to establish any standard for "appropriate medical screening," a principal obligation under the statute, can only mean that it did not intend to establish a federal standard of care regarding the quality of the screening rather than the fact of screening. Similarly, refusal to stabilize a known emergency medical condition goes to the question of pro-

viding any treatment rather than treatment that may have constituted malpractice.

The United States' textual argument also comes up short because it cannot explain the drafting of the statute's provision for administrative sanctions against hospitals and physicians, which applies to those who "*negligently*" violate the Act. 42 U.S.C. § 1395dd(d)(1). If, as the United States argues, the definitions and substantive obligations of EMTALA incorporate a "reasonableness" standard of care, then EMTALA is a classic negligence statute. That is, the statute by definition can only be violated by "negligent" conduct. Under that reading, however, Congress's use of the term "negligently" in the administrative sanctions provision is superfluous and redundant, contrary to the well-established canon of statutory construction that a statute should be read "with the assumption that Congress intended each of its terms to have meaning." *Bailey* v. *United States*, 516 U.S. 137, 145 (1995). See also *United States* v. *Nordic Village, Inc.*, 503 U.S. 30, 36 (1992) (citing "the settled rule that a statute must, if possible, be construed in such fashion that every word has some operative effect"); *Platt* v. *Union Pac. R.R. Co.*, 99 U.S. 48, 58 (1878) ("[T]he admitted rules of statutory construction declare that a legislature is presumed to have used no superfluous words"). Therefore, Congress's use of the phrase "negligently" in the administrative sanctions provision must be read to have some meaning, which necessarily suggests that the substantive provisions of the Act do not already embody a "reasonableness" or "negligence" standard of care.[7]

[7] The United States concedes that EMTALA's private right of action provision "does not itself specify 'negligence' or any other standard to govern such an action." U.S. Br. 21 n.8. It nevertheless argues that it "believe[s] that the EMTALA provisions cited above that define a hospital's obligations in terms of reasonableness make clear that a private suit based on an asserted violation of Section 1395dd(b) must rest *at least* on an allegation of negligence with respect to the duties imposed by those provisions." *Id.* (emphasis added). The government's suggestion that it "believes" EMTALA's private right of action provision imposes a standard of care that is something more than negligence, without making any attempt to identify what that standard of care is, simply demonstrates that EMTALA does not set forth *any* standard of care with "unmistakable" clarity.

Finally, the United States wholly fails to address EMTALA's preemption provision, which provides that "[t]he provisions of this section do not preempt any State or local law requirement, except to the extent that the requirement directly conflicts with a requirement of this section." 42 U.S.C. § 1395dd(f). This preemption provision is narrow and, on its face, gives the widest possible scope to existing state law, which includes the well-established body of state malpractice law. This provision makes plain that Congress did not intend to preempt or displace that body of law by imposing a federal standard of care. By the same token, this provision is hardly consistent with an intent on the part of Congress fundamentally to alter the federal-state balance by creating a federal standard of care that parallels or overlays existing state standards of care. At the very least, this provision provides further evidence that the "clear statement" rule is not satisfied in this case.

Moreover, the legislative history of EMTALA contains no hint that Congress intended to create a federal malpractice statute. The few general statements from the Senate floor debates cited by the United States hardly imply, much less establish, that Congress intended fundamentally to change the federal-state balance by creating a federal malpractice statute. See U.S. Br. 25-26. To the contrary, the United States' anemic discussion of the legislative sources simply highlights the fact that they provide no affirmative support for its sweeping position. Indeed, the United States concedes that the House debates

on COBRA "rarely touched on EMTALA" and "[do] not clarify the instant question." *Id.* at 25 n.10. Congress's silence in the legislative history provides compelling evidence that it did not intend to change fundamental federal-state relations in this traditionally state-based area of tort law. See *Edmonds* v. *Compagnie Generale Transatlantique*, 443 U.S. 256, 266-67 (1979) ("[S]ilence [in legislative history] . . . while contemplating an important and controversial change in existing law is unlikely"); *Bass*, 404 U.S. at 350 ("[T]he legislative history provides scanty basis for concluding that Congress faced these serious questions and meant to affect the federal-state balance").

To the extent the snippets quoted by the government offer any useful insights, they support respondent's position that EMTALA is a gap-filling statute, i.e., the law merely imposes duties that were not imposed under state law at the time. Thus, Senator Durenberger characterized the problem in terms of a hospital "turn[ing] its back on an emergency medical situation." 131 Cong. Rec. 28,568 (1985). Similarly, Senator Dole described the hospital's duty as a "responsibility of providing an adequate first response to a medical crisis. . . ." *Id.* at 28,569. There is nothing in these expressions of intent by the bill's floor manager and co-sponsor to indicate that EMTALA would be a federal malpractice law that enforces continuing diagnosis and treatment responsibilities months after a patient's admission and regardless of whether the attending physicians' professional judgment is unquestionably that the patient no longer suffers from an emergency medical condition. Those are matters thoroughly regulated by state law and, contrary to the Solicitor General's suggestion, EMTALA does not clearly supersede the States' common law in these areas.

Respondent therefore urges the Court to follow the unanimous decisions of the courts of appeals that have held that EMTALA does not fundamentally alter the

federal-state balance in this traditional area of state concern by implementing a federal standard of care for hospitals that provide emergency medical treatment. These courts also correctly have recognized that interpreting EMTALA as a federal negligence or malpractice statute for hospitals would have enormous practical consequences. It effectively would convert every common law malpractice claim against a hospital concerning emergency medical treatment into an EMTALA claim that could be brought, in federal court, without regard to the amount in controversy. This would lead to a huge increase in federal court litigation involving private rights of action under EMTALA, as suggested by the already sizable docket of court of appeals decisions involving civil suits under EMTALA, most of which involved situations where the Secretary of Health and Human Services did not choose to pursue administrative sanctions and the courts ultimately dismissed the claims on the ground that they simply were malpractice claims that should be raised under state law. In sum, if the United States' position were to prevail, hospitals would be uniquely subject to malpractice claims in federal court, even where essentially the same claims could be brought and historically have been brought in state court.

Interpreting EMTALA as a negligence statute also would dramatically change its nature and scope, and divorce it from Congress's focus on patient dumping. Under the various limiting principles applied by the courts of appeals, federal courts largely have avoided any inquiry into, or second-guessing of, the substance of medical judgments made by hospitals and physicians. If, however, EMTALA imposes a federal standard of care for medical diagnosis and treatment, such inquiries would be unavoidable. Indeed, federal courts would have to develop a substantive federal common law of medical malpractice in the context of emergency treatment. This would effectively transform the Act from one aimed at

preventing the problem of patient dumping to one concerned with punishing hospitals and physicians for substantive errors in the physicians' diagnosis and treatment of emergency room patients. EMTALA civil suits would therefore involve the same expert-intensive, after-the-fact inquiries into the soundness of medical decisions that characterize malpractice suits brought in state courts. As the courts of appeals have recognized, this expansive role for EMTALA cannot be squared with the narrow anti-dumping concerns that motivated Congress to enact the statute. Accordingly, this Court should reject the United States' interpretation.

II. ALTHOUGH THE COURT OF APPEALS' MOTIVE TEST LACKS A CLEAR STATUTORY ANCHOR, THE COURT SHOULD CONSIDER ALTERNATIVE GROUNDS FOR AFFIRMANCE THAT WILL REINFORCE THE UNIFORM RULE IN THE COURTS OF APPEALS THAT EMTALA SHOULD BE CONSTRUED NARROWLY TO PRESERVE STATE LAW REGULATING THE QUALITY OF MEDICAL CARE IN HOSPITALS.

If this Court rejects the United States' position, the question remains as to how a hospital's duties under EMTALA should be defined to ensure that the statute does not become a vehicle for raising mere malpractice claims in federal court. That was certainly the goal of the Sixth Circuit both in this case and in *Cleland*, and to that end, the court of appeals incorporated a "motive" test as part of the private cause of action under EMTALA. Under this view a hospital could be liable only if its refusal to provide a screening examination or to provide appropriate medical care to stabilize a patient's emergency condition were improperly motivated, either by concerns for the patient's indigence or by "prejudice against the race, sex, or ethnic group of the patient; distaste for the patient's condition (e.g., AIDS patients); personal dislike or antagonism between the medical personnel and the

patient; disapproval of the patient's occupation; or political or cultural opposition." Pet. App. at A9-A10 (quoting *Cleland*, 917 F.2d at 272).

At the end of the day, respondent cannot defend the judgment on this ground. The statute, by its terms, does not require proof of motive. Instead, the language of the statute, albeit not a model of clarity, is couched in objective terms that are inconsistent with implying a subjective motive standard. The legislative history does not clearly support this approach and the Secretary of HHS has rejected the interpretation. The principal argument that supports this rule is that Congress did not intend to supersede state law when it enacted EMTALA and a motive requirement preserves state law to the maximum extent feasible.

The court's goal is laudatory and its conclusion is consistent with principles of federalism, but its method is not one that Congress adopted and no good purpose would be served by attempting to defend that interpretation. Nevertheless, for reasons that follow, it is absolutely plain that the summary judgment entered in respondent's favor in this case would have been entered on this identical record in virtually every other circuit court of appeals, but on either of two different legal theories, both of which were pressed below. Neither of these theories was addressed because *Cleland* was the law of the Sixth Circuit, but it is plain that, if the Sixth Circuit had rejected either of the alternative theories, then such a decision would warrant review by this Court for essentially the same reason this case was appropriate for review—a split in the circuit courts of appeals on a recurring legal issue of federal law. Accordingly, nothing prevents this Court from considering both of these issues now.[8]

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[8] This Court repeatedly has held that the prevailing party may "defend its judgment on any ground properly raised below whether

Moreover, compelling reasons exist to entertain alternative grounds for affirmance in this particular setting. The Court has been particularly inclined to entertain alternative grounds where the issues are squarely presented and addressing them would serve the interest in efficient judicial administration. In *Blonder Tongue Labs., Inc.* v. *University of Illinois Found.*, 402 U.S. 313, 320-21 n. 6 (1971), for example, the Court addressed an issue not raised in the petition for certiorari because the issue was "a significant one" and "[had] been fully briefed and argued by the parties." Those factors are present here. For the Court to vacate and remand on the motive issue will hardly achieve anything because the court of appeals, which clearly is in search of a limiting principle for EMTALA, will almost certainly adopt one of the two alternative approaches outlined below. For this Court to address either or both issues would certainly conserve scarce judicial resources. The issue is posed and petitioner can present her arguments in response in her reply brief. If she requires additional pages or time, respondent certainly would have no opposition to either request.

More broadly, there are few statutes that have generated nearly so many court of appeals decisions all in search of limiting principles to ensure that EMTALA does not become a litigation-creating machine. Each of those courts has struggled to discern a textually-based rationale for accomplishing Congress's objective of ending the practice of patient dumping without divesting States of their traditional regulatory authority.

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or not that ground was relied upon, rejected, or even considered by the District Court or the Court of Appeals." *Washington* v. *Yakima Indian Nation*, 439 U.S. 463, 476 n.20 (1979). Respondent properly raised both of the proposed alternative grounds in the courts below. *See infra*, nn. 9 & 12. In any event, the Court has inherent authority to consider issues that were not presented in the petition for certiorari and not raised in the courts below. *Vance* v. *Terrazas*, 444 U.S. 252, 258-59 n.5 (1980); *Dandridge* v. *Williams*, 397 U.S. 471, 475-76 n.6 (1976).

While the Court certainly expected to decide only the narrow motive issue posed by petitioner, this is the first case under EMTALA that this Court has chosen to review and the case provides a significant opportunity for the Court to examine the statute completely and to adopt an interpretation that achieves Congress's objectives without allowing EMTALA to expand beyond its appropriate boundaries. Nor is respondent offering these alternative rationales in a vacuum. It is the United States that has suggested the Court might use this case broadly to opine about the nature of the duties hospitals must assume under EMTALA by accepting federal monies. The government's position should be rejected, and the most effective way to halt the inevitable creep that this statute will undergo if HHS's enforcement efforts are left unattended is for this Court to declare that either or both alternative interpretations offered below in support of the judgment are correct. Accordingly, the Court should consider respondent's alternative grounds for affirmance in the unusual circumstances posed by this case. As respondents demonstrate now, the courts of appeals have derived at least two limiting principles from the language, structure, and purpose of the statute. This a suitable vehicle for the Court to recognize either or both of these limiting principles, each of which provides a clear alternative ground for affirming the judgment below.

III. THE JUDGMENT BELOW SHOULD BE AFFIRMED BECAUSE RESPONDENT WAS UNDER NO DUTY TO STABILIZE MS. JOHNSON SINCE HER TREATING PHYSICIANS DID NOT HAVE ACTUAL KNOWLEDGE THAT SHE WAS SUFFERING FROM AN EMERGENCY MEDICAL CONDITION AT THE TIME THEY ORDERED HER TRANSFER.

Several courts of appeals, including the court below, have held that EMTALA does not impose a duty on hospitals to stabilize a patient under 42 U.S.C. § 1395dd(b), or to stabilize a patient prior to transfer under 42 U.S.C.

§ 1395dd(c), unless the treating physicians have actual knowledge that the patient has an emergency medical condition that is in an unstable state. See *Marshall* v. *East Carroll Parish Hosp.*, 134 F.3d 319, 325 (5th Cir. 1998); *Vickers* v. *Nash Gen. Hosp., Inc.*, 78 F.3d 139, 145 (4th Cir. 1996); *Summers* v. *Baptist Med. Ctr. Arkadelphia*, 91 F.3d 1132, 1140 (8th Cir. 1996); *Eberhardt* v. *City of Los Angeles*, 62 F.3d 1253, 1259 (9th Cir. 1995); *Urban by and Through Urban* v. *King*, 43 F.3d 523, 525-27 (10th Cir. 1994); *Holcomb* v. *Monahan*, 30 F.3d 116, 117 (11th Cir. 1994); *Gatewood* v. *Washington Healthcare Corp.*, 933 F.2d 1037, 1041 (D.C. Cir. 1991); *Cleland* v. *Bronson Health Care Group, Inc.*, 917 F.2d 266, 271 (6th Cir. 1990) ("If the emergency nature of the condition is not detected, the hospital cannot be charged with failure to stabilize a known emergency condition"). This argument has clear support in the plain language of the statute, is consistent with the concerns that Congress articulated in the legislative history, and properly distinguishes actions under EMTALA from actions under state malpractice and negligence statutes. Because the undisputed evidence in this case is that Ms. Johnson's treating physicians unanimously concluded that she was stable and did not have an emergency medical condition at the time of her transfer, this theory provides a clear alternative ground for affirming the judgment below.[9]

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[9] This argument was properly preserved in the courts below. Respondent argued to the district court in its motion for summary judgment that "[a] necessary prerequisite of the hospital's duty to stabilize is the hospital's knowledge of the patient's emergency medical condition, gleaned from the medical screening." Memorandum in Support of Defendant Galen of Virginia, Inc. d/b/a University of Louisville Hospital's Motion for Partial Summary Judgment ("Respondent's Summary Judgment Memorandum"), R. 63, at 7 (citing *Cleland*). *See also id.* ("Plaintiff's burden is to show that the Hospital knew that Ms. Johnson had an emergency medical condition on the date of her discharge in order to show that the Hospital breached its duty to stabilize prior to her transfer to a nursing home"); *id.* at 8 ("The hospital's duty to stabilize arises only after it ascertains from the initial screening that an individual *has an emergency medical condition*") (emphasis in original).

Respondent raised this same argument on appeal. In its Sixth Circuit brief, respondent quoted the holding of the court in *Vickers* v. *Nash Gen. Hosp.*, 78 F.3d 139, 145 (4th Cir. 1996), that "'a hospital must actually perceive the seriousness of the medical condition and nevertheless fail to act to stabilize it.'" Brief of Appellee, at 15. Respondent then argued that "[i]n this case, Johnson's treating physicians 'perceived' that Johnson was stable, and believed that the transfer to Crestview was appropriate." *Id.* at 16.

The duty to stabilize a patient under 42 U.S.C. § 1395dd(b) applies when an individual comes to a hospital "and the hospital *determines* that the individual has an emergency medical condition." (emphasis added). The most natural reading of this statutory language is that the duty to stabilize a patient cannot arise unless the treating physicians at the hospital actually diagnose or detect the emergency medical condition, *i.e.*, unless the hospital has actual knowledge of the medical emergency. See *Vickers*, 78 F.3d at 145; *Summers*, 91 F.3d at 1140; *Urban*, 43 F.3d at 526; *Gatewood*, 933 F.2d at 1041. Once the hospital becomes aware of an emergency condition, however, the stabilization duty is unequivocally triggered and the hospital is required to act on its knowledge by stabilizing the patient. If the hospital fails to stabilize a *known* emergency condition, i.e., the hospital acts in conscious disregard of its knowledge of an emergency medical condition, then EMTALA's stabilization requirement is violated. Similarly, because the transfer requirements of 42 U.S.C. §§ 1395dd(c) apply only when a patient "has an emergency medical condition which has not been stabilized" within the meaning of § 1395dd(b), the transfer requirements are not triggered unless the hospital has actual knowledge of an emergency condition that requires stabilization.

The court in *Vickers* explained that this reading of the statutory language properly distinguishes a claim under EMTALA from a malpractice claim:

> On its face, [42 U.S.C. § 1395dd(b)] takes the actual diagnosis as a given, only obligating hospitals to stabilize conditions that they actually detect. . . . The Act does not hold hospitals accountable for failing to stabilize conditions of which they were not aware, or even conditions of which they should have been aware. [citation omitted] EMTALA would otherwise become coextensive with malpractice claims for negligent treatment.

78 F.3d at 145. The principle that the physician's actual diagnosis must be taken "as a given" under EMTALA identifies the critical distinction between EMTALA and state malpractice actions. Under the plain language of the statute, EMTALA imposes a duty on hospitals to treat patients when a physician "determines" or has actual knowledge that the patient has an emergency condition, not when a physician merely "should have known" that the patient has an emergency condition.

This interpretation of the statutory language is most consistent with the anti-dumping concerns that motivated Congress to enact the statute. If the physician actually knew that the patient had an emergency medical condition, and acted in disregard of that knowledge by refusing to stabilize the patient, then the hospital engaged in "dumping" and violated EMTALA. If, instead, the physician did not know but arguably "should have known" of the emergency medical condition, this may be a matter for state malpractice law, but is not "dumping" and hence does not constitute a violation of EMTALA. In short, Congress addressed its concerns about patient "dumping" by enacting a statute that provides a cause of action against hospitals when physicians refuse to treat patients whom they *know* have emergency medical conditions, not when they merely make mistakes and fail to diagnose such conditions in the first instance.[10]

[10] The actual knowledge requirement imposed by a majority of the courts of appeals is consistent with the fact that Congress required "negligence" for the imposition of administrative sanctions by the Secretary of Health and Human Services, but did not require negligence under the private right of action provision. For purposes of determining whether an injured patient is entitled to compensation, it makes sense to impose liability when a physician knows of an emergency medical condition and fails to act on this knowledge. For purposes of determining whether administrative sanctions against individual physicians and/or hospitals are appropriate, however, Congress reasonably concluded that carelessness would be an appropriate inquiry. In any event, it is difficult to draw comparisons between the administrative sanctions and private right of action provisions of EMTALA because they were designed to address different actors.

Under an actual knowledge test, the hospital's motive for failing to treat a patient can be relevant for purposes of establishing liability, but proof of an improper motive is not *required*. Motive evidence, when it exists, can be highly probative and helpful to the trier of fact. For example, if a plaintiff presents evidence that the physician was aware of the patient's emergency medical condition, but refused to treat it *because the patient was indigent*, this would establish a classic instance of dumping and hence a clear violation of EMTALA. Thus, motive evidence can play a role in establishing liability because it can shed light on the question whether a hospital acted in disregard of actual knowledge of a medical emergency. But proof of an improper motive is not *required* under EMTALA. The essence of a violation is failure to stabilize a known emergency medical condition. Once that has been established, the trier of fact need not determine the hospital's precise motive for failing to treat the patient.

Finally, as the court noted in *Urban*, an actual knowledge requirement does not create an incentive for physicians to "deliberately fail to diagnose emergency medical conditions in an effort to avoid liability under [EMTALA]." 43 F.3d at 526. EMTALA expressly requires hospital emergency rooms to perform appropriate medical screenings. *Id.* at 527. In addition, physicians and hospitals can be held liable under state malpractice laws if physicians fail properly to diagnose patients. *Id.*

Since EMTALA does not supplant or affect state malpractice laws, it does not alter physicians' powerful incentives to diagnose patients correctly.

There is no dispute in this case that Ms. Johnson's physicians unanimously concluded that she was stable and did not have an emergency medical condition at the time of her transfer to Crestview. Her treating physicians each submitted an affidavit stating that, in their respective medical opinions, Ms. Johnson was stable when she was transferred to Crestview. J.A. 131 (Affidavit of David Richardson, M.D. ("Richardson Aff.") ¶ 5); J.A. 133 (Affidavit of Walid Abou-Jaoude, M.D. ("Abou-Jaoude Aff.") ¶ 4); J.A. 135 (Affidavit of Brian Harbrecht, M.D. ("Harbrecht Aff.") ¶ 4). See also J.A. 103 (Miller Dep. at 30) ("In my opinion, she did not have an emergency medical condition on the date of her transfer"). Moreover, none of Ms. Johnson's doctors ever considered delaying the transfer because of the suspected urinary tract infection. J.A. 130 (Abou-Jaoude Dep. at 53). Indeed, even the doctor who treated Ms. Johnson at Midwest Hospital conceded that her condition when she arrived at Midwest was "substantially worse" than when she had been discharged from Humana under physician's orders. Stuy Dep. at 76, 91-92. This testimony is uncontroverted, and makes plain that Ms. Johnson's physicians did not diagnose or have actual knowledge of an emergency medical condition prior to her transfer that gave rise to a duty to stabilize. Accordingly, Ms. Johnson's EMTALA claim must fail as a matter of law.[11]

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11 The findings by the courts below that a genuine issue of material fact exists with regard to whether Ms. Johnson's condition was "stable" at the time of her transfer is legally irrelevant for at least two reasons. First, the question is what the physician's actual knowledge or perception was. Thus, whether that assessment was correct or not is just not "material" under EMTALA. Second, the question is not as both the courts below and the Solicitor General put it, whether Ms. Johnson was in a stable condition. The question is whether she had an "emergency condition" that required continued treatment. Here, again, the record is uncontroverted that Ms. Johnson's infections were not uncommon and that they did not preclude a transfer to a skilled nursing facility. In sum, the question whether Ms. Johnson's condition was in fact stable is simply not relevant to any legal issue under EMTALA.

IV. THE JUDGMENT BELOW SHOULD BE AFFIRMED BECAUSE EMTALA DOES NOT APPLY TO TREATMENT AND DISCHARGE DECISIONS OCCURRING AFTER A PATIENT'S INITIAL SCREENING AND STABILIZING TREATMENT.

At least two courts of appeals have held that EMTALA's duty to stabilize a patient only applies to emergency medical conditions that are diagnosed at the time of the patient's initial screening in the emergency room. Once those emergency medical conditions have been stabilized, the requirements of EMTALA are wholly satisfied and the statute does not impose any obligations with respect to further conditions or complications that may arise after the hospital admits the patient for treatment. After a patient's initial screening and stabilizing treatment, the hospital's duty to the patient is governed solely by state law. *Bryan* v. *Rectors and Visitors of Univ. of Va.*, 95 F.3d 349 (4th Cir. 1996); *Collins* v. *DePaul Hosp.*, 963 F.2d 303 (10th Cir. 1992). This interpretation of EMTALA is supported by the plain language of the statute and its legislative history, and is consistent with principles of federalism. This argument provides a clear alternative ground for affirming the judgment below because it is undisputed that Ms. Johnson was properly screened by respondent as having an emergency medical condition in the form of life-threatening trauma injuries, admitted to the hospital, and stabilized for that condition through surgery and other treatment.[12]

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12 This argument was properly preserved in the courts below. Respondent argued in the district court in its motion for summary judgment that "EMTALA applies first and foremost to a patient who comes to the hospital seeking treatment for an emergency medical condition." Respondent's Summary Judgment Memorandum,

In *Bryan*, the court rejected the EMTALA claim of a patient who was diagnosed by a hospital emergency room as having an emergency medical condition in the form of respiratory distress, admitted to the hospital, and treated for twelve days. At that time, the hospital determined through its internal procedures that no further efforts should be made to prevent the patient's death, which occurred eight days alter. 95 F.3d at 350. The court of appeals rejected the EMTALA claim brought by the patient's estate on the ground that the hospital had satisfied the requirements of EMTALA by admitting the patient and providing stabilizing treatment for 12 days. The court held that

> the stabilization requirement was intended to regulate the hospital's care of the patient only in the immediate aftermath of the act of admitting her for emergency treatment and while it considered whether it would undertake longer-term full treatment or instead transfer the patient to a hospital that could and would undertake that treatment.

Id. at 352; see also *id.* ("The stabilization requirement is thus defined . . . without any reference to the patient's long-term care within the system"). The court explained that

> [o]nce EMTALA has met that purpose of ensuring that a hospital undertakes stabilizing treatment for a patient who arrives with an emergency condition, the patient's care becomes the legal responsibility of the hospital and the treating physicians. And, the legal adequacy of that care is then governed not by EMTALA but by the state malpractice law that

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at 4; *id.* ("Congress' purpose in enacting EMTALA was to ensure 'that patients with medical emergencies would receive *emergency care*'") (quoting *Thornton* v. *Southwest Detroit Hosp.*, 895 F.2d 1131, 1132 (6th Cir. 1990)) (emphasis added). Respondent cited the same language from *Thornton* in its brief on appeal. Brief of Appellee, at 8.

everyone agrees EMTALA was not intended to preempt.

Id. at 351. *Accord Collins*, 963 F.2d at 307 (rejecting EMTALA claim on the ground that the hospital complied with EMTALA by diagnosing the patient as having an emergency medical condition, placing him in the intensive care unit, and treating him for 26 days).

The standard applied by the Fourth and Tenth Circuits find clear support in the plain language of the statute. The duty to provide screening under 42 U.S.C. § 1395dd(a) applies only when a hospital "has a hospital emergency department" and an individual "*comes to the emergency department*" and requests an examination or treatment. (emphasis added). The duty to stabilize a patient under 42 U.S.C. § 1395dd(b) applies only when an individual "*comes to a hospital* and the hospital determines that the individual has an emergency medical condition." (emphasis added). Congress's specific references to the patient's initial arrival at the hospital strongly suggest that it meant for EMTALA to apply only when a patient suffering from a possible emergency condition makes initial contact with a hospital and attempts to obtain an evaluation and treatment. The primary concern of EMTALA, in other words, is emergency medical care, not every treatment and discharge decision that is made in the hospital setting simply because the patient happened to have been admitted initially into an emergency room.

The legislative history strongly supports this interpretation. Members of Congress repeatedly declared that the core evil addressed by the legislation is patient "dumping" by hospital *emergency rooms*, i.e., the practice of some emergency rooms of refusing *emergency medical care* to patients who cannot pay. See, e.g., H.R. Rep. No. 99-241, pt. 1, at 27 (1985), *reprinted* in 1986 U.S. Code Cong. & Admin. News, 42, 605 ("The [House Ways and Means] Committee is greatly concerned about the increaing number of reports that *hospital emergency rooms* are

refusing to accept or treat patients *with emergency conditions* if the patient does not have medical insurance") (emphasis added); 131 Cong. Rec. S 13,904 (daily ed. Oct. 23, 1985) ("We must put an end to certain unsafe practices, often referred to as 'patient dumping,' whereby a hospital, for purely financial reasons, *refuses to initially treat or stabilize an individual with a true medical emergency*") (statement of Sen. Dole) (emphasis added); *id.* ("Under the provision of this amendment, a hospital is charged *only with the responsibility of providing an adequate first response to a medical crisis*") (statement of Sen Dole) (emphasis added); 131 Cong. Rec. H 9503 (daily ed. Oct. 31, 1985) ("[I]t should be the national policy for hospitals and free standing emergency centers to provide high quality *emergency care* to all patients without discriminating on the grounds of economic status, color, race, religion, sex or national origin") (statement of Rep. Bilirakis) (emphasis added).

This reading of EMTALA is consistent with principles of federalism because it proceeds from the notion that EMTALA was not meant to duplicate state malpractice laws that govern established doctor-patient relationships, but instead was meant to fill a gap in state tort law, which generally does not impose on hospitals an obligation to treat patients in the first instance. As the *Bryan* court noted, EMTALA's "core purpose is to get patients into the system who might otherwise go untreated and be left without a remedy because traditional medical malpractice law affords no claim for failure to treat." 95 F.3d at 351. See also *Gatewood*, 933 F.2d at 1041 (EMTALA "is not intended to duplicate preexisting legal protections, but rather to create a new cause of action, generally unavailable under state tort law, for what amounts to failure to treat"); *Brooks* v. *Maryland Gen. Hosp., Inc.*, 996 F.2d 708, 710 (4th Cir. 1993) (Congress enacted EMTALA to require hospitals to provide universal emergency care because "[u]nder traditional state tort law, hospitals are under no legal duty to provide this care").

This reading of EMTALA also is consistent with principles of federalism because it rejects the notion that EMTALA creates an unbounded federal duty on hospitals to provide long-term care to patients based on an as-yet-to-be developed national standard of care. As the *Bryan* court reasoned:

> Under [plaintiff's] interpretation, every presentation of an emergency patient to a hospital covered by EMTALA obligates the hospital to do much more than merely provide immediate, emergency stabilizing treatment with appropriate follow-up. Rather, without regard to professional standards of care or the standards embodied in the state law of medical malpractice, the hospital would have to provide treatment indefinitely—perhaps for years—according to a novel, federal standard of care derived from the statutory stabilization requirement. We do not find this reading of the statute plausible.

95 F.3d at 351. The test applied by the Fourth and Tenth Circuits limits the federal cause of action to decisions concerning the initial screening and treatment of patients with possible emergency conditions. It avoids "federalization" of the entire panoply of treatment and discharge decisions made by physicians caring for patients in hospitals, which include difficult and complex decisions concerning the transfer of patients, such as Ms. Johnson, who have severe and chronic conditions that can only be improved by rehabilitation in non-hospital settings.

Under the standard applied by the Fourth and Tenth Circuits, the judgment below plainly was correct. There is no dispute in this case that Ms. Johnson's physicians properly diagnosed the life-threatening trauma injuries that she presented in the emergency room, promptly admitted her to the hospital, and stabilized those injuries during her two month hospital stay. The uncontradicted testimony of her doctors is that she was "stable" throughout July and was moved to a "step-down unit" within the hospital. J.A. 116-18 (Abou-Jaoude Dep. at 15, 17); see also *id.* at 123

(Ms. Johnson was "stable" during the two weeks prior to her discharge). Moreover, Ms. Johnson's doctors initiated procedures to transfer her to a skilled nursing facility to begin the rehabilitation process because they concluded that she was "stable" and "making progress." J.A. 118-20 (Abou-Jaoude Dep. at 19-21) and Abou-Jaoude Dep. at 18, 22. Indeed, petitioner has never disputed that Ms. Johnson's initial injuries were stabilized during her two-month hospital stay. Petitioner's claim is that Ms. Johnson developed a *new* infection prior to her transfer that rendered her unstable. Under the test employed by the Fourth and Tenth Circuits, however, that new complication is irrelevant because EMTALA does not apply to the treatment and transfer decisions made by Ms. Johnson's physicians after her initial emergency condition was stabilized. Accordingly, under that test, the summary judgment plainly was appropriate.[13]

* * * *

If this case had arisen in any Circuit (other than the Sixth) in which the scope of EMTALA has previously been decided, the outcome would have been exactly the same as the decision below. That is, the hospital would have been awarded summary judgment and the patient would have been left to assert whatever state law malpractice claim she has. Of course, the reasoning of the other courts would have been markedly different. Those courts would have held that the undisputed lack of actual knowledge by the hospital and its physicians of any emergency medical condition at the time of transfer and the undisputed provi-

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[13] The district court's finding that there was a genuine issue of material fact as to whether Ms. Johnson was stabilized at the time of her transfer to Crestview was based on conflicting testimony concerning the effects of the new infection that she acquired the day before the transfer. See Pet. App. A29-A36. Under the test articulated in *Bryan* and *Collins*, however, that finding is irrelevant because EMTALA did not apply to the treatment and transfer decisions made by Ms. Johnson's physicians at that time. *See supra*, n.11.

sion of treatment to stabilize the emergency medical condition for which Ms. Johnson was initially admitted to the hospital required dismissal of Ms. Johnson's claims under EMTALA.

The Sixth Circuit reached the same ultimate judgment, but by a path that cannot be located within the language of the statute. Respondent urges the Court to do more than simply inform the court of appeals of the error of its way, and instead, take advantage of this case as an opportunity to make clear that EMTALA is not a radical statute that effectively supersedes state malpractice law for all patients who arrive at a hospital's emergency room.

The Court should hold that EMTALA fills the gap in state law by creating a physician-patient relationship that imposes upon hospitals limited duties to respond to the known needs of a patient's immediate emergency condition and then looks to state law exclusively to govern the physician-patient relationship thereafter. By taking this approach, the Court will accomplish much more than merely eliminating an aberrational rule of law that applies only in four States, which is what petitioner urges the Court to do. Instead, the Court can provide critical guidance that will spare both hospitals (and physicians) and patients the need to expend scarce resources litigating the metes and bounds of EMTALA and spare federal courts the burden they now assume of being inundated with routine medical malpractice cases masquerading as federal statutory causes of action.

The argument of the United States as *amicus curiae* nudges the Court to go beyond the limited objectives of petitioner. Respondent thus urges the Court to recognize the provocative nature of the government's submission and to reject it by entertaining and adopting respondent's alternative theories in support of the judgment below.

CONCLUSION

For the foregoing reasons, the judgment of the court of appeals should be affirmed.

Respectfully submitted,

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